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RESEARCH ARTICLE

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Personal psychedelic experience of psychedelic therapists during training: should it be required, optional, or prohibited?

Daniel Villiger

Uehiro Centre for Practical Ethics, University of Oxford, Oxford, UK

ABSTRACT

Personal psychedelic experience is common among psychedelic therapists and often considered to be a necessary aspect of training: only personal psychedelic experience allows psychedelic therapists to properly guide patients through their own psychedelic experience, to truly understand that experience, and to help them integrate it into their lives. But is this really true? The present paper examines the value of therapists' personal psychedelic experience, why this value may be higher than that of personal experience with other psychotropic drugs, and whether it justifies a requirement of personal psychedelic experience for psychedelic therapists. The analysis, which also considers the literature on therapists' personal experience with the mental disorder being treated or with psychotherapy, concludes that the current evidence does not justify making personal psychedelic(-like) experience a requirement for psychedelic therapists. However, because therapists' personal psychedelic experience can be valuable to both therapists and patients, and because the likelihood of harm is very low, psychedelic therapists should be given the opportunity to have a psychedelic experience during their training.

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Introduction

Psychedelics have made an impressive comeback in mental health care. In the last decade, an increasing number of studies have investigated the therapeutic effects of classic serotonergic psychedelics, including lysergic acid diethylamide (LSD), psilocybin, and N,N-dimethyltryptamine (DMT).[1] The present results are promising and suggest efficacy in the treatment of major depressive disorder (Carhart-Harris, Bolstridge, et al., 2016; Carhart-Harris et al., 2018, 2021; Goodwin et al., 2022; Palhano-Fontes et al., 2019; Raison et al., 2023; Reiff et al., 2020; Rotz et al., 2023; Sanches et al., 2016), anxiety with and without a life-threatening illness (Gasser et al., 2014; Griffiths et al., 2016; Holze et al., 2023; Johnson & Griffiths, 2017; Reiff et al., 2020; Ross et al., 2016), and alcohol and smoking cessation (Bogenschutz et al., 2015; Garcia-Romeu et al., 2019; Johnson, Garcia-Romeu, & Griffiths, 2017; Johnson, Garcia-Romeu, Johnson, et al., 2017; Krebs & Johansen, 2012). Meta-analyses and systematic reviews underscore the potential benefits of psychedelics in mental health care and suggest that psychedelics can be safely used in the clinical context (Andersen et al., 2021; Bender & Hellerstein, 2022; Haikazian et al., 2023; Ko et al., 2023; Maia et al., 2022; Romeo et al., 2020; Roscoe & Lozy, 2022; Simonsson et al., 2023; Wheeler & Dyer, 2020).

Among the classic serotonergic psychedelics, psilocybin is currently receiving the most attention. In February 2024, there were 84 ongoing clinical studies using psilocybin, including four phase 3 trials (ClinicalTrials, 2024). For DMT and LSD, there were seven and four ongoing clinical trials, respectively. In addition to their use in clinical trials, psychedelics have begun to be used clinically in non-research contexts too. Countries such as Canada, Switzerland, and Israel grant psychiatrists special permission to use psychedelics under certain conditions, and in 2023 Australia has become the first country to generally allow psilocybin for treatment (Ducharme, 2023; Haridy, 2023). Taken together, we can already observe significant clinical use of psychedelics, and it is very likely that this use will continue to increase in the coming years.

CONTACT Daniel Villiger daniel.villiger@philosophy.ox.ac.uk Uehiro Centre for Practical Ethics, Faculty of Philosophy, University of Oxford, Suite 1 Littlegate House, St Ebbe's Street, Oxford OX1 1PT, UK
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